



County of Santa Clara

Emergency Medical Services System

ADMINISTRATIVE ORDER

Number:	AO 2025-005
Issued:	March 29, 2025
Title:	Changes to Policy 602: 911 EMS Patient Destination and Policy 605: Prehospital Trauma Triage
Effective:	April 1, 2025 @ 0000

I. Declaration

The Santa Clara County Emergency Medical Services (EMS) Agency has determined that changes to current policy are required. Consistent with *Santa Clara County Prehospital Care Policy 109: Policy Development and Implementation*, the EMS Director, or designee, may issue Administrative Orders when immediate changes are necessary.

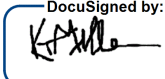
II. Statement of Change and Rationale

The EMS Agency has updated Policy 602: 911 EMS Patient Destination and Policy 605: Prehospital Trauma Triage due to the designation of Regional Medical Center as an Adult Trauma Center and STEMI Receiving Center as of April 1, 2025 at 0000. Policy 602 Schedule A will be deleted. All Trauma Alert Patients will be transported to the closest appropriate Adult or Pediatric Trauma Center by either distance or time of transport.

Field providers should monitor EMResource to confirm availability of specialty services prior to making a transport destination decision.

III. Execution

Administrative Order # 2025-005 is in effect as of **April 1, 2025 @ 0000**.

DocuSigned by:

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 Ken Miller, MD, PhD 3/29/2025
 EMS Medical Director

Signed by:

 05A74EB0479941F...
 Nicholas Clay, 3/29/2025
 EMS Agency Director

Please direct any questions to Mike Cabano, EMS Assistant Director, by phone at 408.794.0625, or via email at michael.cabano@ems.sccgov.org



911 EMS PATIENT DESTINATION

Effective: April 1, 2025
Replaces: March 3, 2025

Deleted: March 3

Deleted: August 12, 2024

I. Purpose

The purpose of this policy is to ensure that all patients who require emergency ambulance service are transported to the approved facility most appropriate for their emergency medical condition.

II. Patient's Choice

- A. Patients shall be transported to their facility of choice if the requested facility meets the requirements of this policy and regardless of their ability to pay.
- B. During times of EMS System surge, patients may be assigned to hospital destinations and may not be able to select a specific destination. Examples of EMS System surge include, but are not limited to, multiple patient events (Multiple Casualty Incident Plan), activation of County Routing, states of local emergency, etc.

III. Routine Patient Destination

- A. The routine patient destination is a basic or comprehensive emergency department in an acute care hospital, or the emergency department of a federally owned and operated hospital as identified in *Table B: Approved In-County Services*.
- B. Patients are to be transported to a routine patient destination unless the patient meets the "In-Extremis", "Specialty Care", or "Special Circumstances" patient destination criteria identified in this policy.
- C. If no patient preference, routine patients shall be transported to:
 - 1. The closest hospital to the incident location as determined by the total emergency ambulance transport time; **and**
 - 2. That is accepting emergency ambulance patients as is identified in *Table B: Approved In-County Services*.

IV. In-Extremis Patient Destination

- A. A patient that is determined to be "In-Extremis" presents with a condition that benefits most from immediate emergency ambulance transportation to the closest hospital.

B. An In-Extremis patient is defined as a patient that presents with one or more of the following:

1. A breech presentation or protruding limb during a delivery.
2. A visible external bleed that cannot be controlled by EMS personnel where significant blood loss continues to occur despite the use of direct pressure and/or application of a tourniquet.
3. The inability to be ventilated adequately following the use of appropriate basic and advanced airway adjuncts and procedures.

C. In-Extremis patients shall be transported to the hospital that is:

1. The closest to the incident location as determined by total emergency ambulance transport time; **and**
2. That is **not** on internal disaster.

V. Psychiatric Patients

A. Patients not on a Welfare and Institutions Code, Section 5150 hold:

1. Psychiatric patients in need of medical evaluation shall be provided a medical care assessment in accordance with Policy #700-S04 or Policy #700-S05 and should be transported to an emergency department identified in Table B: Approved In-County Services.
2. If a psychiatric patient's medical complaint requires immediate attention, the psychiatric patient should be transported to the closest emergency department/specialty care center identified in Table B: Approved In-County Services.

B. Patients on a Welfare and Institutions Code (WIC), Section 5150 hold:

1. Psychiatric patients who have an associated medical presentation or injury shall be provided a medical care assessment in accordance with Policy #700-S04 or Policy #700-S05, shall be transported to the closest appropriate emergency department, not on bypass, as identified in Table B: Approved In-County Services.
 - a. If the patient meets Specialty Care criteria, they shall be transported to the closest, appropriate, open Specialty Care Center, as identified in Table B: Approved In-County Services.
2. Psychiatric patients who do not have an associated medical presentation or injury shall be transported to the destination written on the WIC 5150 Form.
 - a. If the patient meets the criteria in 700-X05, and Santa Clara Valley Medical Center is written as the destination, the patient shall be transported to Emergency Psychiatric Services (EPS).

- b. If no destination is identified prior ambulance arrival, and the patient presents with isolated psychiatric complaints (necessitating the WIC 5150-5185), it should be encouraged for the destination to be EPS.
- C. Psychiatric patients with no medical complaint that are being transported by a BLS ambulance, may be transported to the destination established by the authorized writer responsible for executing the 5150 hold.

VI. Specialty Care Destination

- A. In some circumstances, the most appropriate facility is one that offers specialized services based on the EMS providers primary impression of the patient's condition and based on the criteria of this policy.
- B. The "most appropriate hospital" for specialty care patients is the hospital that is:
 - 1. The closest to the incident location as determined by total emergency ambulance transport time; **and**
 - 2. Is designated to provide the specialty service desired; **and**
 - 3. Is accepting emergency ambulance patients.
- C. **Trauma Alert Patients** – Patients that meet High Risk Injury Pattern, Mental Status, and/or Vital Sign (Red Criteria) or Moderate Risk Mechanism of Injury or EMS Judgement (Yellow Criteria) according to *Santa Clara County Prehospital Care Policy 605: Prehospital Trauma Triage*.
 - 1. Injured patients that do not meet trauma alert criteria shall be transported to a destination prescribed by Section III: Routine Patient Destination.
 - 2. Adult Trauma Alert Patients shall be transported to the closest open Adult Trauma Center, identified in Table B: Approved In-County Services, determined by distance or time of travel.
 - 3. Pediatric Trauma Alert Patients shall be transported to the closest open Pediatric Trauma Center, identified in Table B: Approved In-County Services, determined by distance or time of travel.
 - 4. Pregnant trauma alert patients more than twenty (20) weeks gestation are to be transported to the closest trauma center with an approved Level III Neonatal ICU (Stanford Hospital or Santa Clara Valley Medical Center).
 - 5. In addition to the provisions of *Santa Clara County Prehospital Care Policy #603: Hospital Bypass*, if **all** Trauma Centers are not accepting emergency ambulance patients, the patient shall be transported to the:
 - a. Closest emergency department to the incident location as determined by total emergency ambulance transport time; **and**
 - b. That is accepting emergency ambulance patients.

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POLICY # 605

D. **Burn Patients** – Patients meeting major burn criteria as per *Santa Clara County Prehospital Care Policy #605: Prehospital Trauma Triage* shall be transported to the Burn Center at Santa Clara Valley Medical Center (VMC) via the Trauma Center.

1. If Santa Clara Valley Medical Center (VMC) is on trauma bypass, the patient meeting major burn criteria shall be transported to VMC as the closest burn center, regardless of trauma bypass.
2. Patients meeting major burn criteria and having additional concurrent trauma, and if the traumatic injuries pose the greater risk of morbidity or mortality, shall be transported to:
 - a. The closest trauma center identified in *Table B: Approved In-County Services* to the incident location by the total emergency ambulance transport time; **and**
 - b. That is accepting emergency ambulance patients.

E. **Stroke Alert Patients**

1. Patients that are identified as meeting Comprehensive Stroke Alert Criteria according to *Santa Clara County Prehospital Care Policy #700-A13: Stroke* shall be transported to:
 - a. The closest Comprehensive Stroke Center identified in *Table B: Approved In-County Services* to the incident location as determined by total emergency ambulance transport time; **and**
 - b. That is accepting emergency ambulance patients that meet stroke alert criteria.
 - c. If the transport time to the closest Comprehensive Stroke Center is greater than forty-five (45) minutes, transport to the closest Primary Stroke Center in accordance with Section (F)(2) of this policy.
2. Patients that are identified as meeting Primary Stroke Alert Criteria according to *Santa Clara County Prehospital Care Policy #700-A13: Stroke* shall be transported to:
 - a. The closest Stroke Center identified in *Table B: Approved In-County Services* to the incident location as determined by total emergency ambulance transport time; **and**
 - b. That is accepting emergency ambulance patients that meet stroke alert criteria.

F. **STEMI Alert Patients** – Patients that are identified as meeting STEMI Alert Criteria according to *Santa Clara County Prehospital Care Policy #700-A08: Chest Pain - Suspected Cardiac Ischemia* shall be transported to:

1. The closest STEMI Receiving Center identified in *Table B: Approved In-County Services* to the incident location as determined by total emergency ambulance transport time; **and**
 2. That is accepting emergency ambulance patients that meet STEMI Alert.
- G. **ROSC (Return of Spontaneous Circulation)** – Adult Patients achieving ROSC of cardiac etiology according to *Santa Clara County Prehospital Care Policy #700-A07: Cardiac Arrest* shall be transported to:
1. The closest STEMI Receiving Center identified in *Table B: Approved In-County Services* to the incident location as determined by total emergency ambulance transport time; **and**
 2. That is accepting emergency ambulance patients that meet ROSC criteria.
- H. **Pediatric Patients** – Patients that are less than 15 years of age
1. A pediatric patient who meets the critically ill criteria listed below:
 - a. Cardiac dysrhythmia
 - b. Evidence of poor perfusion (pallor, cyanosis, etc.)
 - c. Severe respiratory distress (retractions, stridor, etc.)
 - d. Persistent altered mental status
 - e. Stroke like symptoms
 - f. Status epilepticus
 - g. Brief Resolved Unexplained Event (BRUE)
 - h. Return of Spontaneous Circulation (ROSC)
 - i. Suspicion of child abuse
 - j. Paramedic discretion

Shall be transported to:

 - a. The closest Advanced Pediatric Receiving Center, identified in *Table B: Approved In-County Services*, to the incident location as determined by total emergency ambulance transport time; **and**
 - b. That is accepting emergency ambulance patients.
 2. A pediatric patient who does **not** meet trauma, in-extremis, or critically ill criteria as listed above shall be transported to (paramedic discretion also

applies):

- a. Any Pediatric Receiving Center of choice as identified in *Table B: Approved In-County Services*; **and**
- b. That is accepting emergency ambulance patients.

VII. Special Circumstances

- A. In some situations, special circumstances may have a direct relation to the selection of the most appropriate transport destination. Within this section, "County" shall mean the EMS Duty Chief, EMS Agency, County Medical-Health Branch or any other position or function designated by the EMS Agency.
- B. **EMS System Surge / Multiple Patients Events**
 1. When Central Patient Routing is in effect, all patient destination assignments will be directed by the County through County Communications.
 2. Patients may be transported to acute care hospitals not listed in this policy only when directed by the County.
 3. If out of county mutual aid ambulance are being used in the Santa Clara County Operational Area, unless directed by the County, all transports will occur in accordance with the destinations prescribed in this policy.
 4. When authorized and as directed by the County, patients may be transported to Alternate Care Sites (ACS), Field Treatment Sites (FTS), or Mobile Field Hospitals (MFH).
- C. **Base Hospital Directed Destination** – Patients may be transported to any in-county destination authorized within this policy when directed by the Base Hospital.
- D. **Stanford Health Care** - Stanford Hospital has two Emergency Department entrances (Table E). All adult patients should be transported to 500P. All patients > 20 weeks pregnant in active labor (regardless of maternal age) and all pediatric patients (less than 15 years of age) should be transported to 300P.
- E. **EMS Air Resource Destination** – Patient destination shall be determined by the ground crew and provided to the air crew.
 1. If the pilot believes that flight to the selected destination is unsafe, the flight crew will advise the ground crew and a destination will be determined collaboratively and according to the direction provided in this policy.
 2. When a ground crew is not present or if the ground crew is not designated EMS providers, the flight crew shall determine destination based on this policy.

F. Transport to out of county Hospitals from Santa Clara County

1. Patients may be transported to out of county hospitals only when permitted by this policy.
2. Santa Clara County prehospital care personnel shall determine and honor out of county 911 ambulance diversion statuses prior to beginning transport.
3. Santa Clara County prehospital care personnel shall notify the out of county hospital prior to beginning transport of the patient to ensure that the hospital is open and receiving patients.

G. Destination Changes While Transporting

1. A patient may change their requested destination at any time if the ambulance has not arrived on hospital property and the requested destination meets the requirements of this policy.
2. If the ambulance has arrived on the property of an acute care hospital, patient care shall be transferred to the staff of such hospital.
3. If enroute to a hospital, the patient wishes to leave the ambulance, EMS personnel shall:
 - a. Attempt to convince the patient to continue to the selected destination or to the closest emergency department able to receive emergency ambulance patients.
 - b. If the patient continues to wish to leave the ambulance, stop the ambulance as soon as it is safe to do so and permit the patient to leave once in a safe location and then immediately notify their communications center.
 - c. EMS providers will make efforts to keep patient safe and out of harm's way, consistent with *Santa Clara County Prehospital Care Policy #618: EMS Life Safety Procedures*.

H. Custodial Patients from the County Jails

1. The preferred destination for patients that are in custody and are from incidents occurring in the County Jail (San Jose), Elmwood (Milpitas), and Juvenile Hall (San Jose), is Santa Clara Valley Medical Center, if the transport is consistent with the directives contained in this policy. For example, the hospital must be accepting emergency ambulance patients. The destination is determined by jail staff, not the patient.
2. From time to time the EMS Agency will issue specific transport policies related to ambulance transport of high risk/high profile inmates. These policies will augment the direction provided within this policy.

- I. **Incidents Occurring at Acute Care Hospitals** – If an acute care hospital is the incident location, patients shall be transported to the emergency department of the incident location hospital, except:
1. When the acute care hospital is not accepting emergency ambulance patients.
 2. When multiple patients require transport and must be distributed to multiple acute care hospitals.
 3. When the acute care hospital requests a 911 emergency ambulance response to transport patient in accordance with *Santa Clara County Prehospital Care Policy #620: Interfacility Transfer-Ground Ambulance*.
- J. **Veterans** – Patients that are veterans may be transported to the Palo Alto Veterans Administration Hospital (PAV) if:
1. Requested by the patient; **and**
 2. If transport is consistent with the directives contained within this policy.
- K. **Ventricular Assist Devices (VAD)** - VAD centers in Santa Clara County are located at Kaiser Santa Clara Hospital and Stanford Health Care.
1. If a VAD patient presents with stroke symptoms, these two stroke/VAD centers would be the preferred destinations.
 2. A VAD patient visiting from outside of the Santa Clara County area should be transported to either one of these two VAD centers for device-related problems.
 3. In traumatic injury of a VAD patient consider a trauma center as a destination since the patient will be anticoagulated.
- L. **Sexual Assault Forensic Examiners (SAFE)** – SAFE centers in Santa Clara County are located at Stanford Health Care (SUH), Santa Clara Valley Medical Center (VMC), and Saint Louise Regional Medical Center (SLH).
1. The preferred destination for patients seeking medical care after sexual assault and intimate partner violence are SLH, SUH or VMC. Patients may be transported to one of these facilities if transport is consistent with the directives contained within this policy.

Table A: Approved Santa Clara County Facilities

Facility	City	ID
El Camino Hospital of Los Gatos	Los Gatos	LGH
El Camino Hospital of Mountain View	Mountain View	ECH
Good Samaritan Hospital	San Jose	GSH
Kaiser Foundation San Jose	San Jose	STH
Kaiser Foundation Santa Clara	Santa Clara	KSC
O'Connor Hospital	San Jose	OCH
Palo Alto Veterans Administration Hospital	Palo Alto	PAV
Regional Medical Center of San Jose	San Jose	RSJ
Saint Louise Regional Medical Center	Gilroy	SLH
Santa Clara Valley Medical Center	San Jose	VMC
Stanford Health Care	Palo Alto	SUH

Table B: Approved In-County Services

	Approved In-County Services											
	EPS	ECH	GSH	KSC	LGH	OCH	PAV	RSJ	SLH	STH	SUH	VMC
Emergency Department (Adult)		✓	✓	✓	✓	✓	✓	✓	✓	✓	✓	✓
Burn Center												✓
Primary Stroke Center		✓	✓	✓	✓	✓		✓	✓	✓	✓	✓
Comprehensive Stroke Center		✓	✓	✓				✓			✓	
STEMI Center		✓	✓	✓		✓		✓		✓	✓	✓
Adult Trauma Center								✓			✓	✓
Pediatric Trauma Center											✓	✓
Advanced Pediatric Center				✓							✓	✓
General Pediatric Center		✓	✓	✓		✓		✓	✓	✓	✓	✓
*Psychiatric Facility	✓	✓	✓	✓	✓	✓	✓	✓	✓	✓	✓	✓
*VAD Center				✓							✓	
*Labor & Delivery		✓	✓	✓	✓	✓			✓	✓	✓	✓
*SAFE Center									✓		✓	✓
*Helipads			✓	✓				✓	✓		✓	✓

***Note:** Santa Clara County EMS Agency does not designate/regulate helipads, L&D, SAFE, VAD, or Psychiatric Centers, this is intended for supplemental information use only.

Table C: Approved Out-of-County Facilities

Facility	City / County	ID
Dominican Hospital	Santa Cruz / Santa Cruz	DOM
Kaiser Foundation Fremont	Fremont / Alameda	KFF
Kaiser Foundation Redwood City	Redwood City / San Mateo	KRC
Sequoia Hospital	Redwood City / San Mateo	SEQ
Washington Township Hospital	Fremont / Alameda	WTH
Watsonville Community Hospital	Watsonville / Santa Cruz	WCH

Table D: Approved Out-of-County Services

	Approved Out of County Services					
	DOM	KFF	KRC	SEQ	WCH	WTH
Emergency Department	☒	☒	☒	☒	☒	☒
*Helipads	☒					☒

***Note:** Santa Clara County EMS Agency does not designate/regulate helipads, this is intended for supplemental information use only.

Table E: Stanford Health Care Ambulance Entrances

Facility	Ambulance Entrance Address
Adult Entrance (500P)	1199 Welch Road
Pediatric Entrance (300P)	900 Quarry Road Extension



911 EMS PATIENT DESTINATION

Effective: April 1, 2025
Replaces: March 3, 2025

I. Purpose

The purpose of this policy is to ensure that all patients who require emergency ambulance service are transported to the approved facility most appropriate for their emergency medical condition.

II. Patient's Choice

- A. Patients shall be transported to their facility of choice if the requested facility meets the requirements of this policy and regardless of their ability to pay.
- B. During times of EMS System surge, patients may be assigned to hospital destinations and may not be able to select a specific destination. Examples of EMS System surge include, but are not limited to, multiple patient events (Multiple Casualty Incident Plan), activation of County Routing, states of local emergency, etc.

III. Routine Patient Destination

- A. The routine patient destination is a basic or comprehensive emergency department in an acute care hospital, or the emergency department of a federally owned and operated hospital as identified in *Table B: Approved In-County Services*.
- B. Patients are to be transported to a routine patient destination unless the patient meets the "In-Extremis", "Specialty Care", or "Special Circumstances" patient destination criteria identified in this policy.
- C. If no patient preference, routine patients shall be transported to:
 - 1. The closest hospital to the incident location as determined by the total emergency ambulance transport time; **and**
 - 2. That is accepting emergency ambulance patients as is identified in *Table B: Approved In-County Services*.

IV. In-Extremis Patient Destination

- A. A patient that is determined to be "In-Extremis" presents with a condition that benefits most from immediate emergency ambulance transportation to the closest hospital.

- B. An In-Extremis patient is defined as a patient that presents with one or more of the following:
 - 1. A breech presentation or protruding limb during a delivery.
 - 2. A visible external bleed that cannot be controlled by EMS personnel where significant blood loss continues to occur despite the use of direct pressure and/or application of a tourniquet.
 - 3. The inability to be ventilated adequately following the use of appropriate basic and advanced airway adjuncts and procedures.
- C. In-Extremis patients shall be transported to the hospital that is:
 - 1. The closest to the incident location as determined by total emergency ambulance transport time; **and**
 - 2. That is **not** on internal disaster.

V. Psychiatric Patients

- A. Patients not on a Welfare and Institutions Code, Section 5150 hold:
 - 1. Psychiatric patients in need of medical evaluation shall be provided a medical care assessment in accordance with Policy #700-S04 or Policy #700-S05 and should be transported to an emergency department identified in Table B: Approved In-County Services.
 - 2. If a psychiatric patient's medical complaint requires immediate attention, the psychiatric patient should be transported to the closest emergency department/specialty care center identified in Table B: Approved In-County Services.
- B. Patients on a Welfare and Institutions Code (WIC), Section 5150 hold:
 - 1. Psychiatric patients who have an associated medical presentation or injury shall be provided a medical care assessment in accordance with Policy #700-S04 or Policy #700-S05, shall be transported to the closest appropriate emergency department, not on bypass, as identified in Table B: Approved In-County Services.
 - a. If the patient meets Specialty Care criteria, they shall be transported to the closest, appropriate, open Specialty Care Center, as identified in Table B: Approved In-County Services.
 - 2. Psychiatric patients who do not have an associated medical presentation or injury shall be transported to the destination written on the WIC 5150 Form.
 - a. If the patient meets the criteria in 700-X05, and Santa Clara Valley Medical Center is written as the destination, the patient shall be transported to Emergency Psychiatric Services (EPS).

- b. If no destination is identified prior ambulance arrival, and the patient presents with isolated psychiatric complaints (necessitating the WIC 5150-5185), it should be encouraged for the destination to be EPS.
- C. Psychiatric patients with no medical complaint that are being transported by a BLS ambulance, may be transported to the destination established by the authorized writer responsible for executing the 5150 hold.

VI. Specialty Care Destination

- A. In some circumstances, the most appropriate facility is one that offers specialized services based on the EMS providers primary impression of the patient's condition and based on the criteria of this policy.
- B. The "most appropriate hospital" for specialty care patients is the hospital that is:
 - 1. The closest to the incident location as determined by total emergency ambulance transport time; **and**
 - 2. Is designated to provide the specialty service desired; **and**
 - 3. Is accepting emergency ambulance patients.
- C. **Trauma Alert Patients** – Patients that meet High Risk Injury Pattern, Mental Status, and/or Vital Sign (Red Criteria) or Moderate Risk Mechanism of Injury or EMS Judgement (Yellow Criteria) according to *Santa Clara County Prehospital Care Policy 605: Prehospital Trauma Triage*.
 - 1. Injured patients that do not meet trauma alert criteria shall be transported to a destination prescribed by Section III: Routine Patient Destination.
 - 2. Adult Trauma Alert Patients shall be transported to the closest open Adult Trauma Center, identified in *Table B: Approved In-County Services*, determined by distance or time of travel.
 - 3. Pediatric Trauma Alert Patients shall be transported to the closest open Pediatric Trauma Center, identified in *Table B: Approved In-County Services*, determined by distance or time of travel.
 - 4. Pregnant trauma alert patients more than twenty (20) weeks gestation are to be transported to the closest trauma center with an approved Level III Neonatal ICU (Stanford Hospital or Santa Clara Valley Medical Center).
 - 5. In addition to the provisions of *Santa Clara County Prehospital Care Policy #603: Hospital Bypass*, if **all** Trauma Centers are not accepting emergency ambulance patients, the patient shall be transported to the:
 - a. Closest emergency department to the incident location as determined by total emergency ambulance transport time; **and**
 - b. That is accepting emergency ambulance patients.

D. **Burn Patients** – Patients meeting major burn criteria as per *Santa Clara County Prehospital Care Policy #605: Prehospital Trauma Triage* shall be transported to the Burn Center at Santa Clara Valley Medical Center (VMC) via the Trauma Center.

1. If Santa Clara Valley Medical Center (VMC) is on trauma bypass, the patient meeting major burn criteria shall be transported to VMC as the closest burn center, regardless of trauma bypass.
2. Patients meeting major burn criteria and having additional concurrent trauma, and if the traumatic injuries pose the greater risk of morbidity or mortality, shall be transported to:
 - a. The closest trauma center identified in *Table B: Approved In-County Services* to the incident location by the total emergency ambulance transport time; **and**
 - b. That is accepting emergency ambulance patients.

E. **Stroke Alert Patients**

1. Patients that are identified as meeting Comprehensive Stroke Alert Criteria according to *Santa Clara County Prehospital Care Policy #700-A13: Stroke* shall be transported to:
 - a. The closest Comprehensive Stroke Center identified in *Table B: Approved In-County Services* to the incident location as determined by total emergency ambulance transport time; **and**
 - b. That is accepting emergency ambulance patients that meet stroke alert criteria.
 - c. If the transport time to the closest Comprehensive Stroke Center is greater than forty-five (45) minutes, transport to the closest Primary Stroke Center in accordance with Section (F)(2) of this policy.
2. Patients that are identified as meeting Primary Stroke Alert Criteria according to *Santa Clara County Prehospital Care Policy #700-A13: Stroke* shall be transported to:
 - a. The closest Stroke Center identified in *Table B: Approved In-County Services* to the incident location as determined by total emergency ambulance transport time; **and**
 - b. That is accepting emergency ambulance patients that meet stroke alert criteria.

F. **STEMI Alert Patients** – Patients that are identified as meeting STEMI Alert Criteria according to *Santa Clara County Prehospital Care Policy #700-A08: Chest Pain - Suspected Cardiac Ischemia* shall be transported to:

1. The closest STEMI Receiving Center identified in *Table B: Approved In-County Services* to the incident location as determined by total emergency ambulance transport time; **and**
 2. That is accepting emergency ambulance patients that meet STEMI Alert.
- G. **ROSC (Return of Spontaneous Circulation)** – Adult Patients achieving ROSC of cardiac etiology according to *Santa Clara County Prehospital Care Policy #700-A07: Cardiac Arrest* shall be transported to:
1. The closest STEMI Receiving Center identified in *Table B: Approved In-County Services* to the incident location as determined by total emergency ambulance transport time; **and**
 2. That is accepting emergency ambulance patients that meet ROSC criteria.
- H. **Pediatric Patients** – Patients that are less than 15 years of age
1. A pediatric patient who meets the critically ill criteria listed below:
 - a. Cardiac dysrhythmia
 - b. Evidence of poor perfusion (pallor, cyanosis, etc.)
 - c. Severe respiratory distress (retractions, stridor, etc.)
 - d. Persistent altered mental status
 - e. Stroke like symptoms
 - f. Status epilepticus
 - g. Brief Resolved Unexplained Event (BRUE)
 - h. Return of Spontaneous Circulation (ROSC)
 - i. Suspicion of child abuse
 - j. Paramedic discretionShall be transported to:
 - a. The closest Advanced Pediatric Receiving Center, identified in *Table B: Approved In-County Services*, to the incident location as determined by total emergency ambulance transport time; **and**
 - b. That is accepting emergency ambulance patients.
 2. A pediatric patient who does **not** meet trauma, in-extremis, or critically ill criteria as listed above shall be transported to (paramedic discretion also

applies):

a. Any Pediatric Receiving Center of choice as identified in *Table B: Approved In-County Services*; **and**

b. That is accepting emergency ambulance patients.

VII. Special Circumstances

A. In some situations, special circumstances may have a direct relation to the selection of the most appropriate transport destination. Within this section, "County" shall mean the EMS Duty Chief, EMS Agency, County Medical-Health Branch or any other position or function designated by the EMS Agency.

B. EMS System Surge / Multiple Patients Events

1. When Central Patient Routing is in effect, all patient destination assignments will be directed by the County through County Communications.
2. Patients may be transported to acute care hospitals not listed in this policy only when directed by the County.
3. If out of county mutual aid ambulance are being used in the Santa Clara County Operational Area, unless directed by the County, all transports will occur in accordance with the destinations prescribed in this policy.
4. When authorized and as directed by the County, patients may be transported to Alternate Care Sites (ACS), Field Treatment Sites (FTS), or Mobile Field Hospitals (MFH).

C. **Base Hospital Directed Destination** – Patients may be transported to any in-county destination authorized within this policy when directed by the Base Hospital.

D. **Stanford Health Care** - Stanford Hospital has two Emergency Department entrances (Table E). All adult patients should be transported to 500P. All patients > 20 weeks pregnant in active labor (regardless of maternal age) and all pediatric patients (less than 15 years of age) should be transported to 300P.

E. **EMS Air Resource Destination** – Patient destination shall be determined by the ground crew and provided to the air crew.

1. If the pilot believes that flight to the selected destination is unsafe, the flight crew will advise the ground crew and a destination will be determined collaboratively and according to the direction provided in this policy.
2. When a ground crew is not present or if the ground crew is not designated EMS providers, the flight crew shall determine destination based on this policy.

F. Transport to out of county Hospitals from Santa Clara County

1. Patients may be transported to out of county hospitals only when permitted by this policy.
2. Santa Clara County prehospital care personnel shall determine and honor out of county 911 ambulance diversion statuses prior to beginning transport.
3. Santa Clara County prehospital care personnel shall notify the out of county hospital prior to beginning transport of the patient to ensure that the hospital is open and receiving patients.

G. Destination Changes While Transporting

1. A patient may change their requested destination at any time if the ambulance has not arrived on hospital property and the requested destination meets the requirements of this policy.
2. If the ambulance has arrived on the property of an acute care hospital, patient care shall be transferred to the staff of such hospital.
3. If enroute to a hospital, the patient wishes to leave the ambulance, EMS personnel shall:
 - a. Attempt to convince the patient to continue to the selected destination or to the closest emergency department able to receive emergency ambulance patients.
 - b. If the patient continues to wish to leave the ambulance, stop the ambulance as soon as it is safe to do so and permit the patient to leave once in a safe location and then immediately notify their communications center.
 - c. EMS providers will make efforts to keep patient safe and out of harm's way, consistent with *Santa Clara County Prehospital Care Policy #618: EMS Life Safety Procedures*.

H. Custodial Patients from the County Jails

1. The preferred destination for patients that are in custody and are from incidents occurring in the County Jail (San Jose), Elmwood (Milpitas), and Juvenile Hall (San Jose), is Santa Clara Valley Medical Center, if the transport is consistent with the directives contained in this policy. For example, the hospital must be accepting emergency ambulance patients. The destination is determined by jail staff, not the patient.
2. From time to time the EMS Agency will issue specific transport policies related to ambulance transport of high risk/high profile inmates. These policies will augment the direction provided within this policy.

- I. **Incidents Occurring at Acute Care Hospitals** – If an acute care hospital is the incident location, patients shall be transported to the emergency department of the incident location hospital, except:
1. When the acute care hospital is not accepting emergency ambulance patients.
 2. When multiple patients require transport and must be distributed to multiple acute care hospitals.
 3. When the acute care hospital requests a 911 emergency ambulance response to transport patient in accordance with *Santa Clara County Prehospital Care Policy #620: Interfacility Transfer-Ground Ambulance*.
- J. **Veterans** – Patients that are veterans may be transported to the Palo Alto Veterans Administration Hospital (PAV) if:
1. Requested by the patient; **and**
 2. If transport is consistent with the directives contained within this policy.
- K. **Ventricular Assist Devices (VAD)** - VAD centers in Santa Clara County are located at Kaiser Santa Clara Hospital and Stanford Health Care.
1. If a VAD patient presents with stroke symptoms, these two stroke/VAD centers would be the preferred destinations.
 2. A VAD patient visiting from outside of the Santa Clara County area should be transported to either one of these two VAD centers for device-related problems.
 3. In traumatic injury of a VAD patient consider a trauma center as a destination since the patient will be anticoagulated.
- L. **Sexual Assault Forensic Examiners (SAFE)** – SAFE centers in Santa Clara County are located at Stanford Health Care (SUH), Santa Clara Valley Medical Center (VMC), and Saint Louise Regional Medical Center (SLH).
1. The preferred destination for patients seeking medical care after sexual assault and intimate partner violence are SLH, SUH or VMC. Patients may be transported to one of these facilities if transport is consistent with the directives contained within this policy.

Table A: Approved Santa Clara County Facilities

Facility	City	ID
El Camino Hospital of Los Gatos	Los Gatos	LGH
El Camino Hospital of Mountain View	Mountain View	ECH
Good Samaritan Hospital	San Jose	GSH
Kaiser Foundation San Jose	San Jose	STH
Kaiser Foundation Santa Clara	Santa Clara	KSC
O'Connor Hospital	San Jose	OCH
Palo Alto Veterans Administration Hospital	Palo Alto	PAV
Regional Medical Center of San Jose	San Jose	RSJ
Saint Louise Regional Medical Center	Gilroy	SLH
Santa Clara Valley Medical Center	San Jose	VMC
Stanford Health Care	Palo Alto	SUH

Table B: Approved In-County Services

Approved In-County Services												
	EPS	ECH	GSH	KSC	LGH	OCH	PAV	RSJ	SLH	STH	SUH	VMC
Emergency Department (Adult)		✓	✓	✓	✓	✓	✓	✓	✓	✓	✓	✓
Burn Center												✓
Primary Stroke Center		✓	✓	✓	✓	✓		✓	✓	✓	✓	✓
Comprehensive Stroke Center		✓	✓	✓				✓			✓	
STEMI Center		✓	✓	✓		✓		✓		✓	✓	✓
Adult Trauma Center								✓			✓	✓
Pediatric Trauma Center											✓	✓
Advanced Pediatric Center				✓							✓	✓
General Pediatric Center		✓	✓	✓		✓		✓	✓	✓	✓	✓
*Psychiatric Facility	✓	✓	✓	✓	✓	✓	✓	✓	✓	✓	✓	✓
*VAD Center				✓							✓	
*Labor & Delivery		✓	✓	✓	✓	✓			✓	✓	✓	✓
*SAFE Center									✓		✓	✓
*Helipads			✓	✓				✓	✓		✓	✓

***Note:** Santa Clara County EMS Agency does not designate/regulate helipads, L&D, SAFE, VAD, or Psychiatric Centers, this is intended for supplemental information use only.

Table C: Approved Out-of-County Facilities

Facility	City / County	ID
Dominican Hospital	Santa Cruz / Santa Cruz	DOM
Kaiser Foundation Fremont	Fremont / Alameda	KFF
Kaiser Foundation Redwood City	Redwood City / San Mateo	KRC
Sequoia Hospital	Redwood City / San Mateo	SEQ
Washington Township Hospital	Fremont / Alameda	WTH
Watsonville Community Hospital	Watsonville / Santa Cruz	WCH

Table D: Approved Out-of-County Services

	Approved Out of County Services					
	DOM	KFF	KRC	SEQ	WCH	WTH
Emergency Department	☒	☒	☒	☒	☒	☒
*Helipads	☒					☒

***Note:** Santa Clara County EMS Agency does not designate/regulate helipads, this is intended for supplemental information use only.

Table E: Stanford Health Care Ambulance Entrances

Facility	Ambulance Entrance Address
Adult Entrance (500P)	1199 Welch Road
Pediatric Entrance (300P)	900 Quarry Road Extension



PREHOSPITAL TRAUMA TRIAGE

Effective: April 1, 2025
Replaces: January 1, 2024

Deleted: January 1, 2024

Deleted: January 1, 2022

I. Purpose

The purpose of this policy is to provide standard criteria for the prehospital triage of trauma and burn patients in Santa Clara County.

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II. Trauma Alert Patient

- A. Trauma alert patients are injured patients who meet ACS National Guidelines for Field Triage Criteria.
- B. Injured patients are to be identified as a Trauma Alert Patient if one or more of the criteria below are met:

High Risk Injury Pattern (Red Criteria):

- A. Penetrating injuries to head, neck, torso, and proximal extremities
- B. Skull deformity, suspected skull fracture
- C. Suspected spinal injury with new motor or sensory loss
- D. Chest wall instability, deformity, or suspected flail chest
- E. Suspected pelvic fracture
- F. Suspected fracture of two or more proximal long bones
- G. Crushed, degloved, mangled, or pulseless extremity
- H. Amputation proximal to wrist or ankle
- I. Active bleeding requiring a tourniquet or wound packing with continuous pressure

Deleted: <#>Adult trauma alert patients are to be transported expeditiously to the closest Trauma Center.¶

¶ Pediatric trauma alert patients under the age of fifteen (15) years old are to be transported to the closest pediatric trauma center (Stanford Hospital or Santa Clara Valley Medical Center).¶

¶ Pregnant trauma alert patients more than twenty (20) weeks gestation are to be transported to the closest trauma center with an approved Level III Neonatal ICU (Stanford Hospital or Santa Clara Valley Medical Center).¶

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Deleted: <#>a

Mental Status and Vital Signs (Red Criteria):

- J. Unable to follow commands (motor GCS <6)
- K. Respiratory rate less than 10 or greater than 29 breaths per minute
- L. Respiratory distress or need for respiratory support
- M. Room-air pulse oximetry less than 90%

N. Vital signs below the following parameters:

1. Age 0-9 years: Systolic BP less than 70 mmHg + 2x age years
2. Age 10-64 years: Systolic BP less than 90 mmHg
3. Age older than 65 years: Systolic BP is less than 110 mmHg
4. Age 10 and older: Heart rate is greater than Systolic BP.

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Moderate Risk Mechanism of Injury (Yellow Criteria):

- O. Auto crash with partial or complete ejection
- P. Auto crash with intrusion > 12 inches occupant site, > 18 inches any site, or need for extrication
- Q. Death in passenger compartment
- R. Child (age 0-9) unrestrained or in unsecure child safety seat
- S. Vehicle telemetry data consistent with severe injury
- T. Rider separated from transport vehicle with significant impact (eg. Motorcycle, ATV, horse etc.)
- U. Rollover with unrestrained occupant
- V. Pedestrian/bicycle rider thrown, run over, or with significant impact
- W. Fall from height > 10 feet (all ages)

Special Considerations (EMS Judgement):

- X. There are other factors that might influence destination which patients should be treated in Trauma Centers. The following should be considered in prehospital trauma triage when a traumatic injury is also present:
 1. Patients with minor traumatic injuries also on anti-coagulants or with bleeding disorders
 2. Time-sensitive extremity injury
 3. EMS provider judgment to transport patient to a trauma center
 4. Hanging/mechanical asphyxiation in cardiac arrest with suspected head or neck injury
 5. Unwitnessed drowning with suspected head or neck injury

6. Low-level falls with significant head impact
7. Any pregnant patient beyond 20 weeks gestation (uterine fundus palpated at or above the umbilicus) in cardiac arrest, that does not meet obvious death criteria

II. Trauma Alert – Ambulance Transport

- A. [Refer to Policy 602: 911 EMS Patient Destination for Adult and Pediatric Trauma receiving centers and destination determination](#)

III. Major Burn Criteria

- A. Patients with burn injuries are to be identified as major burn criteria if any of the following are present:
 1. Partial thickness burns greater than 10% of the total body surface area
 2. Burns that include the face, hands, feet, genitalia, perineum, or major joints
 3. Full thickness burns
 4. Electrical burns, including high voltage (1,000v) and lightning injury
 5. Chemical burns
 6. Inhalation injury
 7. Burn injury in patients with pre-existing medical disorders that could complicate management, prolong recovery, or affect mortality
- B. Transport all identified major burn patients to a designated burn center.
- C. Patients who do not meet major burn criteria should be transported to an appropriate acute care hospital with emergency services.
- D. Patients who meet the major burn criteria and who also meet trauma alert criteria and the traumatic injuries poses a greater risk of morbidity or mortality shall be transported to: (1) the closest trauma center to the incident location by total emergency ambulance transport time: and (2) that is accepting emergency ambulance patients.

Deleted: <#>Transport all Trauma Alerts to a designated Trauma Center.

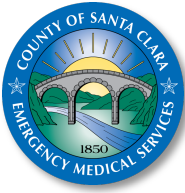
Patients who are not deemed Trauma Alerts according to the criteria established herein should be transported to an appropriate acute care hospital with emergency services.

Pediatric patients who are not deemed Trauma Alerts according to the criteria established herein should be transported to the appropriate Pediatric Receiving Center established in Policy 602

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PREHOSPITAL TRAUMA TRIAGE

Effective: April 1, 2025
Replaces: January 1, 2024

I. Purpose

The purpose of this policy is to provide standard criteria for the prehospital triage of trauma and burn patients in Santa Clara County

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- G. Crushed, degloved, mangled, or pulseless extremity
- H. Amputation proximal to wrist or ankle
- I. Active bleeding requiring a tourniquet or wound packing with continuous pressure

Mental Status and Vital Signs (Red Criteria):

- J. Unable to follow commands (motor GCS <6)
- K. Respiratory rate less than 10 or greater than 29 breaths per minute
- L. Respiratory distress or need for respiratory support
- M. Room-air pulse oximetry less than 90%

- N. Vital signs below the following parameters:
1. Age 0-9 years: Systolic BP less than 70 mmHg + 2x age years
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 3. Age older than 65 years: Systolic BP is less than 110 mmHg
 4. Age 10 and older: Heart rate is greater than Systolic BP

Moderate Risk Mechanism of Injury (Yellow Criteria):

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